

Weight Management Program Bullseye Research

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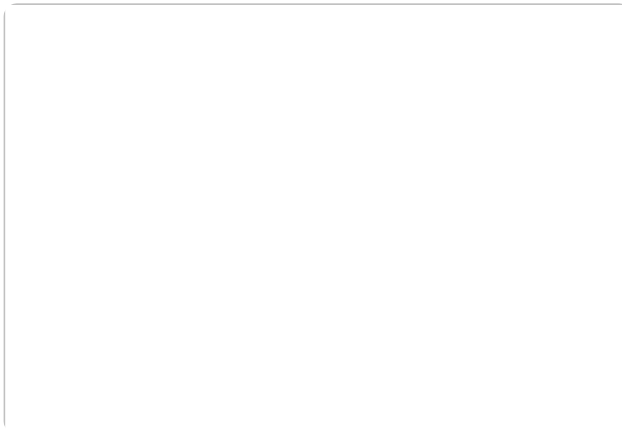
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Method: 1:1 moderated concept interviews following the bullseye customer review process

Audience: Targeted Adults 65+ interested in weight management & healthy aging. 200 applies, anchored to the most aligned target customer based on openness to GLP1s, willingness to work with a new provider, usage of social media (ads) and other factors.

Sample: n = 4 (Usually 5 is best, but had 1 no show)



Why so few interviews? Qualitative research can cover the majority of relevant user topics relatively quickly. Additional research should/can be done test final concept direction.
<https://mitsloan.mit.edu/shared/ods/documents?PublicationDocumentID=5259>

Concepts Tested:

- **Prototype 01:** Lose weight. Stay strong. Strength + muscle preservation.
- **Prototype 02:** Go beyond the scale. Labs, diagnostics, FDA-approved
- **Prototype 03:** Lifestyle reset / tapering. Habits, dashboards, GLP1 off-ramp

Research question: Which value proposition creates the most pull, trust, and urgency for a Medicare-age weight management program?

Results: Value prop hierarchy

Value Prop 1: GLP-1: Safety, Form Factor & Coverage

This was the first decision gate. People did not evaluate the rest of the program until they understood whether the medication felt safe, affordable, and tolerable.

What resonated

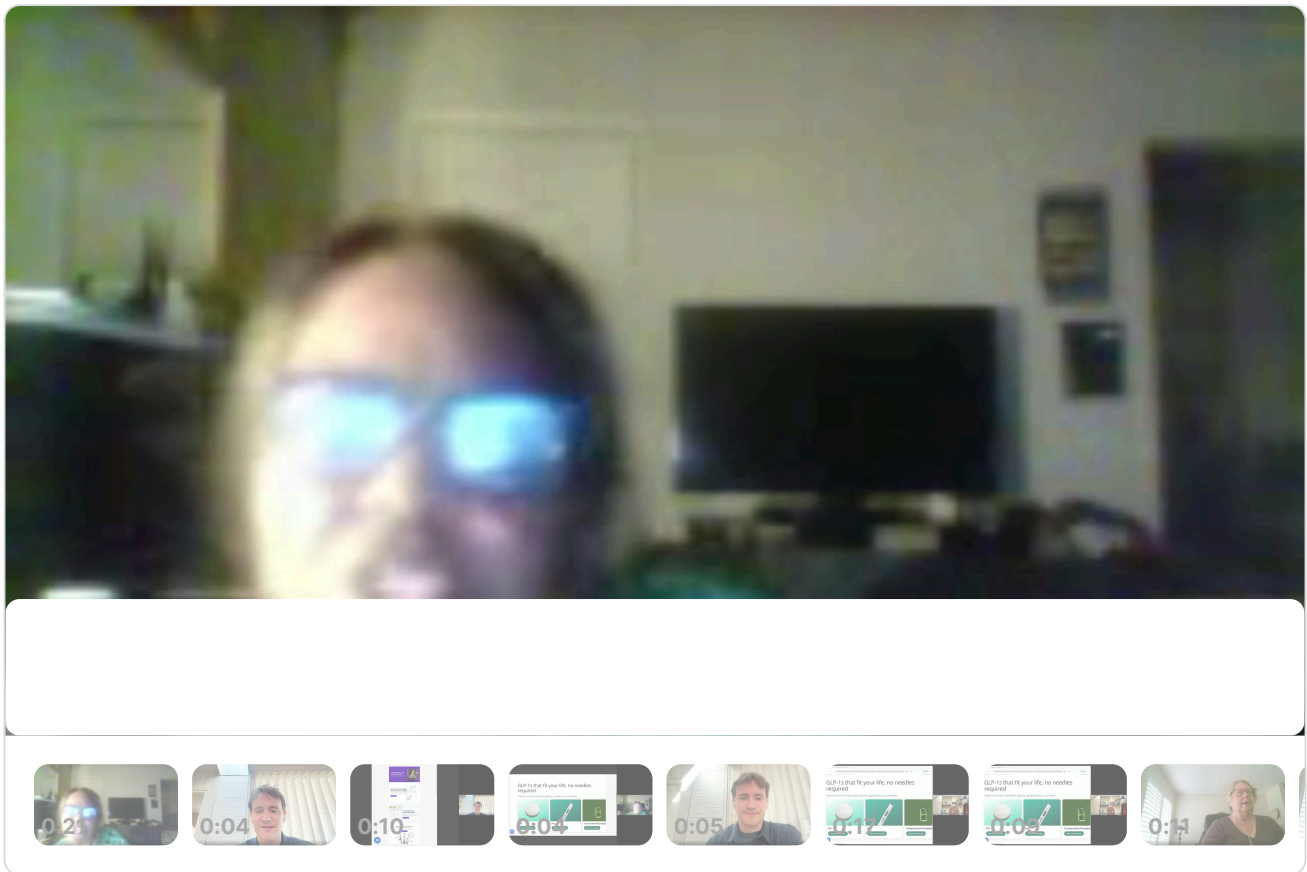
- Pill-first or pill/pen choice.
- FDA-approved medications.
- Clear coverage / affordability.
- Provider oversight and lab testing to avoid side effects.
- Avoiding or managing nausea, constipation, stomach issues, headaches, irritation, and unknown long-term effects.

What to avoid

- Do not lead with compounded medication.
- Do not show a vial/needle as the hero treatment.
- Do not imply "GLP-1 access" without clarifying cost, coverage, safety, and oversight.
- Do not assume "FDA-approved" only creates trust; it also triggers cost/coverage questions.

User need

"Help me access a GLP-1 in a way that feels safe, affordable, and easy for me to take."



Notes:

- Ron ranked barriers clearly: side effects first, cost second; he preferred "pill one, pen two," and rejected vial/needle workflows.
- Linda said she was "deathly afraid of shots" and preferred a pill.
- Jennifer had previously tried semaglutide injections, paid \$400, lost only seven pounds, and stopped because of nausea, constipation, and irritability; she also preferred a pill and rated the pill option highly.
- Robin was concerned about side effects, long-term use, bone density, muscle loss, and compounded semaglutide, and preferred FDA-approved, brand-name drugs.

Value Prop 2: Body Composition: Lean Mass & Bone Density

This was the strongest differentiator after GLP-1 trust. The winning tracking concept was proof that people were losing fat without losing strength, muscle, or bone health.

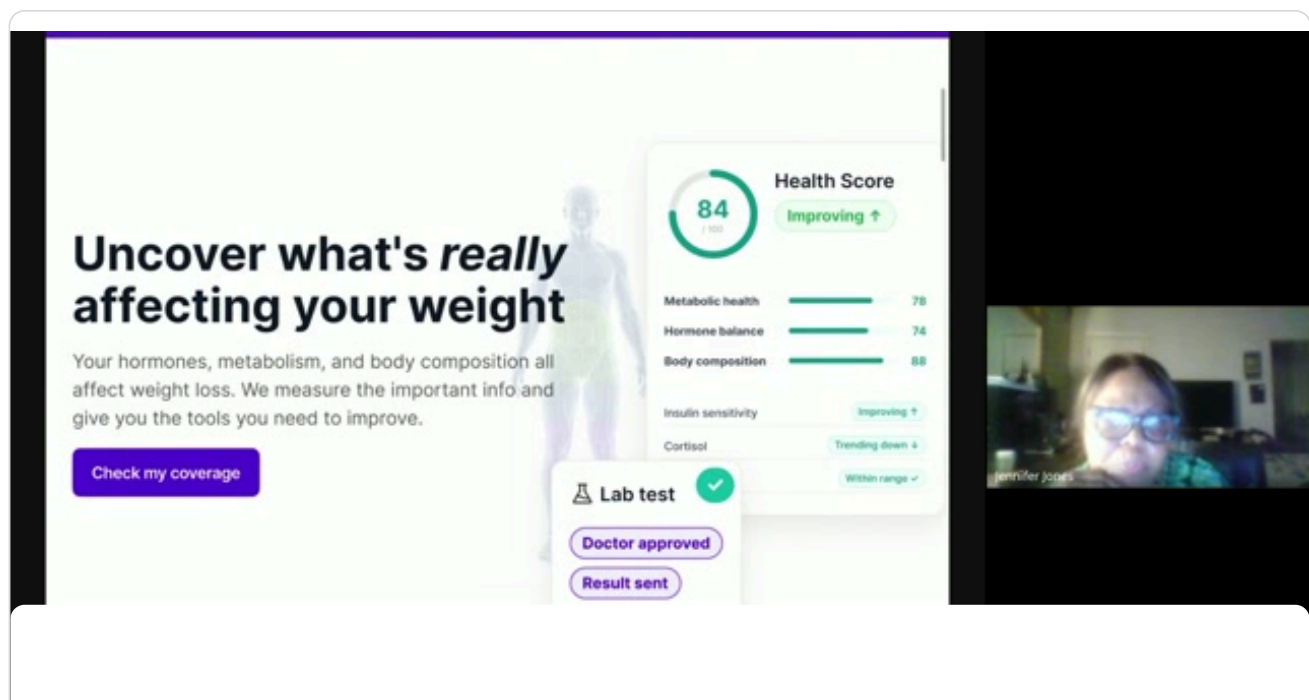
What resonated

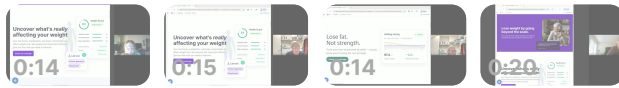
- Lean mass tracking.
- Fat loss vs muscle loss.
- Body composition over scale weight.
- Bone density / osteoporosis was not displayed but mentioned several times
- Knowing whether weight loss is "healthy" weight loss.

What to avoid

- Do not over-index on "metabolic health score" as the main value prop.
- Do not make labs feel like generic PCP bloodwork.
- Do not show body composition tracking without explaining how it works.
- Do not assume people will accept extra devices or special labs without a clear benefit.

User need: "Show me that I'm losing the right kind of weight and that I'm protecting the things that matter as I age."





Notes:

- Jennifer reacted positively to Concept 1 because it combined strength training and weight loss and specifically liked lean mass tracking; she also asked how it would be measured and whether it required a blood draw.
- Jennifer was interested in Concept 2's "health score," but specifically found body composition more relevant than cortisol.
- Robin found lean mass tracking novel and interesting, wanted fat/muscle tracking from Concept 1 combined with the clinical data from Concept 2, and noted that bone density was missing and important.
- Ron's reaction to Concept 2 became very body-composition-specific: he wanted to lose weight in the areas he needed to lose it, while not losing more from already-thin legs.

Value prop 3: Personalized diet and exercise planning that cuts out decision fatigue

Lifestyle support mattered, but patients recoiled at the idea of logging exercises, meals and habits. People wanted help that reduced effort: meal planning, easy choices, seated or balance-safe exercise and accountability.

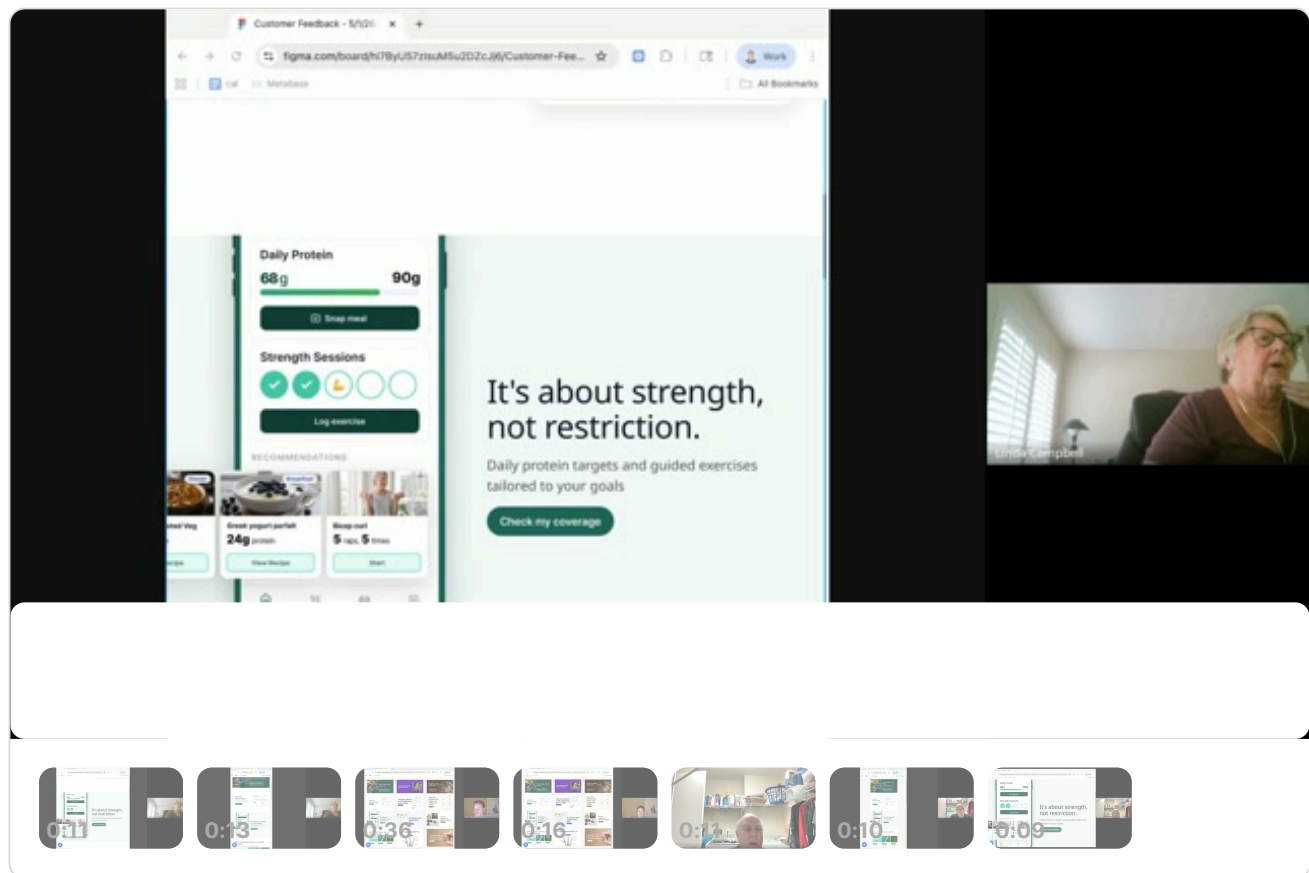
What resonated

- Meal planning before eating, not just logging after.
- Food convenience: menus, prepared foods, restaurant choices, low-effort meals.
- Offering something that doesn't feel restrictive. A lot of patients are familiar with low sugar/keto/atkins because of marketing and/or they are pre-diabetic/diabetic but these diets required too much will power to maintain.
- Photo-based food tracking instead of manual entry, but only for a limited time
- Strength, balance, seated exercise, and age-appropriate movement like seated exercises
- A real person checking in, encouraging, and helping adjust.

What to avoid

- Do not lead with generic habit tracking, it's not differentiated
- Do not require wearables. Even some screens that don't require a wearable (step counting) suggested to the user they would have to wear an unwanted product full time
- Do not show generic exercise that looks inaccessible for older adults.
- Do not overstate chat-based physician access as the value; people wanted oversight/accountability more than constant messaging.

User need: "Make the healthy thing easier to do in my actual life — with food, exercise, and follow-up that fit my age, body, and routine."



Evidence

- Linda did not want to input every meal and exercise session; she wanted a sample diet or meal plan in advance because manual tracking felt too time-consuming.
- Robin liked photo-based meal tracking because manual logging is tedious, and described the real barrier as the time and mental energy of planning, shopping, prepping, cooking, and cleaning.
- Ron was not interested in meal prep; he and his wife often rely on prepared foods, takeout, and simple meals, and he needed seated exercise because of balance limitations.

- Linda's ideal program included a specific meal plan, strengthening and balance exercises, and a mentor/nutritionist for progress and encouragement.

Prototype Readout

Prototype 1 was the most compelling

Prototype 1 was the strongest overall direction because it combined the clearest emotional hook with the most differentiated outcome promise: lose weight, stay strong / lose fat, not strength.

It won because it connected medication, strength, body composition, and aging-specific outcomes. Jennifer selected Concept 1 as the one she would be most likely to sign up for, specifically because of strength training, strength tracking, and a GLP-1 that fits her lifestyle. Ron found Concept 1 most appealing because "strength, not restriction" felt less mentally burdensome and because the pill option fit his existing medication routine. Robin initially found Concept 1 most personally relevant because of strength, muscle, and bone, even though she ultimately wanted Concept 2's clinical layer added.

Prototype 2 addressed concerns about GLP1 safety and side effects, but didn't hook people

Prototype 2 worked best when it explained *why* the program is medically credible: FDA-approved medications, labs, diagnostics, and provider-guided care. But the lab/metabolic-health framing alone did not feel as differentiated, because some participants wondered whether this was different from what a PCP could do. Robin specifically questioned how the lab work differed from routine primary care panels.

The enhanced read is: labs are useful when they support GLP-1 safety, side-effect management, and body-composition outcomes, but patients cared less about metabolic outcomes than they did body composition.

Best parts to preserve

- FDA-approved GLP-1s.
- Labs/diagnostics for safety monitoring.
- Side-effect management.
- Body composition, muscle, bone density, insulin sensitivity.
- Provider-guided interpretation.

What to de-emphasize

- Cortisol / hormone score as the main hook.
- Generic "metabolic health score."
- Nutrition/supplement plan as the lead value prop.

Prototype 3 was the weakest concept

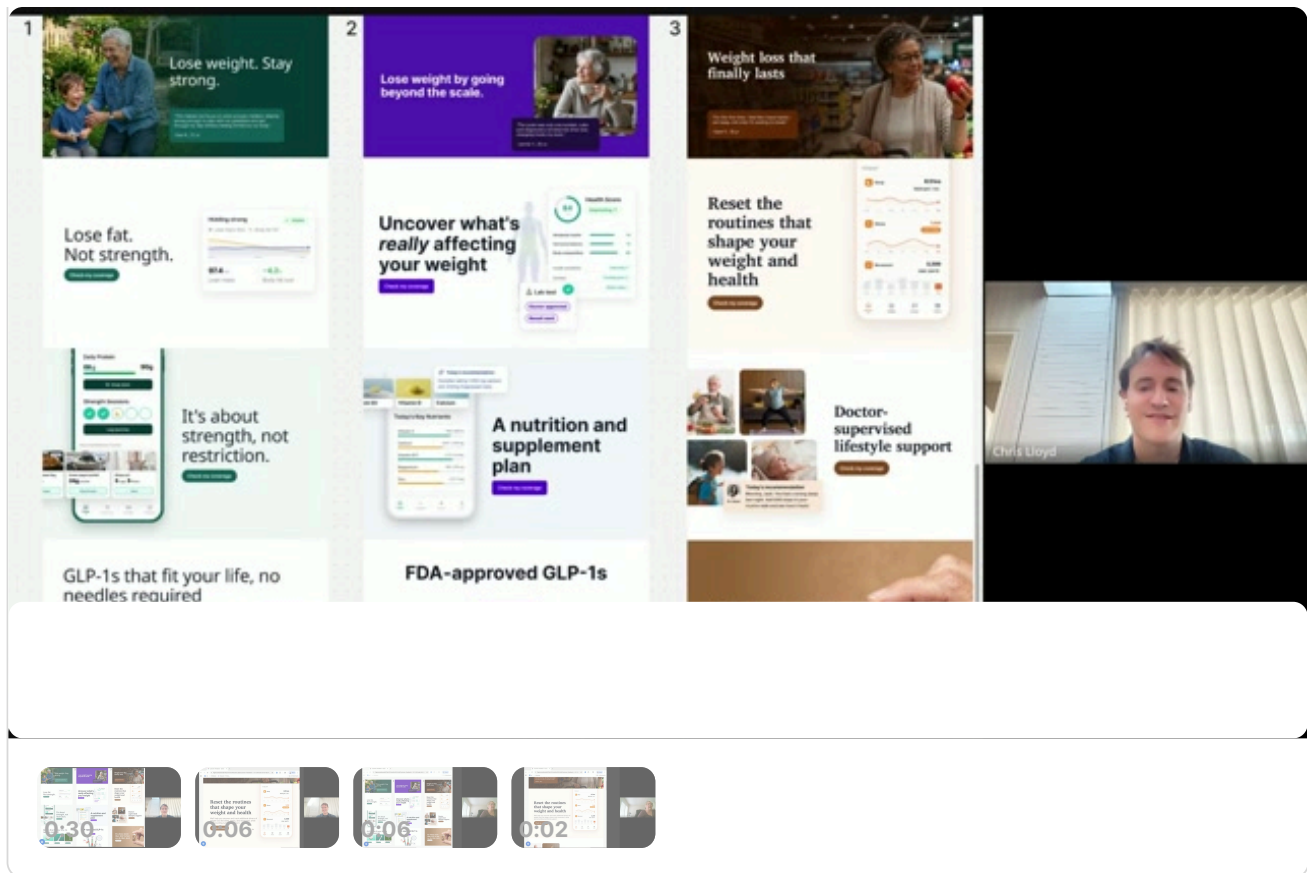
Prototype 3 underperformed as a full concept. It felt more generic, more app-like, and more work. Ron reacted negatively to the 12-month framing as “a lot of work” and rejected the microdose vial/needle format. Robin felt the biometrics tracking overlapped with smartwatches and would not pay for that as the main offer. Linda disliked wearable requirements and did not find app-based physician chat especially useful.

Useful pieces to keep

- Doctor supervision.
- Accountability.
- Tapering / off-ramp logic.
- Lowest effective dose or side-effect-sensitive medication management.

What to avoid

- Wearable-first lifestyle tracking, even screens that suggest the possibility of wearables
- “Reset your routines” as the primary pitch.
- Vial/needle imagery.
- Generic chat coaching.



Winning value props to build around

1. FDA-approved GLP-1 options that fit your life, now covered by Medicare

This should be explicit: pill if appropriate, pen if needed, no scary vial-first imagery, clear coverage, provider oversight.

2. Lose fat, not strength (Marketing message)

This is the best core differentiator. It connects GLP-1s, aging, muscle, body composition, safety, and exercise in one simple promise.

3. Body composition tracking made easy

People liked the idea but immediately asked how it works. The product needs a simple answer: scan, DEXA, smart scale, measurements, labs, or whatever the eventual measurement strategy is. The next brainstorm should be: **how can Bold credibly track muscle/fat/bone-risk over time without making it feel burdensome or expensive?**

4. Older-adult-specific strength and balance plan

Exercise only works if it is clearly personalized and accessible: seated options, balance-safe routines, strength progression, osteoporosis/bone support, joint-friendly options.

5. Convenient diet support, not food homework

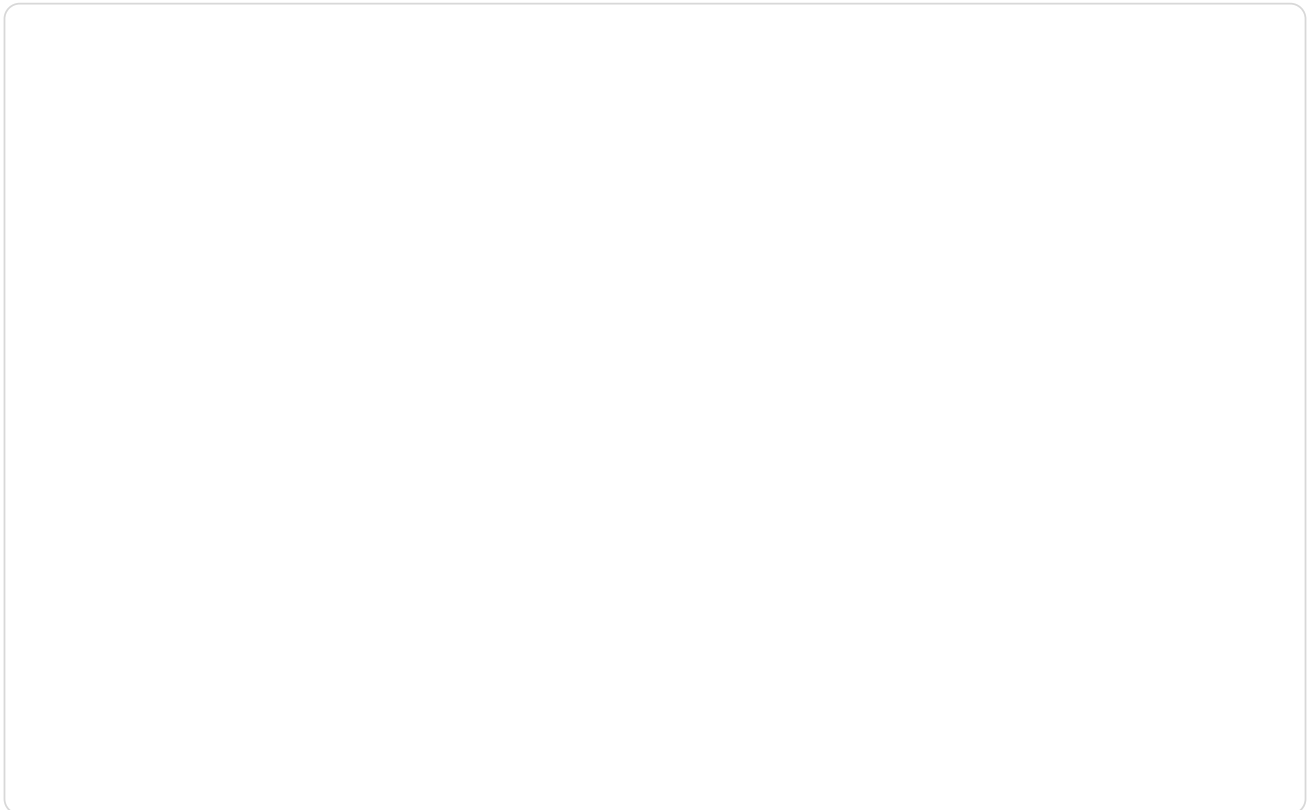
The best food direction is not “track everything.” It is: help me decide what to eat when I am tired, at a restaurant, buying prepared food, or planning meals. Menu scanning / photo-based recommendations / simple meal plans are more compelling than manual logging.

6. Provider oversight as safety + accountability

People did not necessarily want constant physician chat. They wanted someone watching over the program, preventing overmedication/undereating, helping manage side effects, and keeping them accountable.

Target prototype direction

View o





Add an answer block

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4 participants · 3 concepts tested · Jennifer Jones, Ron Riemer, Linda Campbell, Robin English How to read this report Insights are ordered by signal strength — the number of participants who mentioned or reacted to the theme, and the strength of their sentiment. Each insight includes direct quotes and the source session. The final section covers what to deprioritize. ● STRONG SIGNAL

Weight management program bullseye research report